

Available online at [ScienceDirect](https://www.sciencedirect.com)

Resuscitation

journal homepage: www.elsevier.com/locate/resuscitation

Corrigendum

Corrigendum to “European Resuscitation Council Guidelines 2021: Cardiac arrest in special circumstances” [Resuscitation 161 (2021) 152–219]



Carsten Lott^{a,*}, Anatolij Truhlár^{b,c}, Annette Alfonzo^d, Alessandro Barelli^e, Violeta González-Salvado^f, Jochen Hinkelbein^g, Jerry P. Nolan^{h,i}, Peter Paal^j, Gavin D. Perkins^{k,l}, Karl-Christian Thies^m, Joyce Yeung^{k,l}, David A. Zidemanⁿ, Jasmeet Soar^o, the ERC Special Circumstances Writing Group Collaborators¹

^a Department of Anesthesiology, University Medical Center, Johannes Gutenberg-University Mainz, Germany

^b Emergency Medical Services of the Hradec Králové Region, Hradec Králové, Czech Republic

^c Department of Anaesthesiology and Intensive Care Medicine, Charles University in Prague, University Hospital Hradec Králové, Hradec Králové, Czech Republic

^d Departments of Renal and Internal Medicine, Victoria Hospital, Kirkcaldy, Fife, UK

^e Anaesthesiology and Intensive Care, Catholic University School of Medicine, Teaching and Research Unit, Emergency Territorial Agency ARES 118, Rome, Italy

^f Cardiology Department, University Clinical Hospital of Santiago de Compostela, Institute of Health Research of Santiago de Compostela (IDIS), Biomedical Research Networking Centres on Cardiovascular Disease (CIBER-CV), A Coruña, Spain

^g Department of Anaesthesiology and Intensive Care Medicine, University Hospital of Cologne, Cologne, Germany

^h Resuscitation Medicine, University of Warwick, Warwick Medical School, Coventry CV4 7AL, UK

ⁱ Anaesthesia and Intensive Care Medicine, Royal United Hospital, Bath BA1 3NG, UK

^j Department of Anaesthesiology and Intensive Care Medicine, Hospitallers Brothers Hospital, Paracelsus Medical University, Salzburg, Austria

^k Warwick Clinical Trials Unit, Warwick Medical School, University of Warwick, Coventry, UK

^l University Hospitals Birmingham NHS Foundation Trust, Birmingham, UK

^m Department of Anesthesiology, Critical Care and Emergency Medicine, Bethel Medical Centre, OWL University Hospitals, Bielefeld University, Germany

ⁿ Thames Valley Air Ambulance, Stokenchurch, UK

^o Southmead Hospital, North Bristol NHS Trust, Bristol, UK

Page 165 – The last bullet point in the left column should read:

- Resuscitate according to ALS MODIFIED algorithm:
 - VF/ pVT → Defibrillate: apply up to 3 consecutive shocks (<1 min).

- Asystole/extreme bradycardia → Apply early pacing (<1 min).
- PEA → Correct potentially reversible causes. If paced rhythm, turn off pacing to check the underlying rhythm.

DOI of original article: <https://doi.org/10.1016/j.resuscitation.2021.02.011>

* Corresponding author.

E-mail address: lott@uni-mainz.de (C. Lott).

¹ ERC Special Circumstances Writing Group Collaborators are listed in Appendix A.

<https://doi.org/10.1016/j.resuscitation.2021.08.012>

0300-9572/

Page 182 – In the second paragraph, the sentence beginning “isotonic or hypertonic...” should read:

Isotonic or hypertonic fluids should be administered (with blood sodium ≤ 130 mEq/l up to 3×100 ml 3% NaCl at 10 min intervals).²⁰⁶

Page 190 – In the section “Restore pulsatile cardiac rhythm”, the sentence beginning “In the presence of a pulseless...” should read:

In the presence of a pulseless stimulated rhythm, pacing should be paused to eventually unmask the underlying rhythm and follow the appropriate resuscitation protocol.

The authors would like to apologise for any inconvenience caused.